DEPRESSION

Clean disease factsheet | Overview, symptoms, diagnosis, treatment & support

1. Overview

Depression (major depressive disorder) is a common but serious mental health condition that can affect mood, thinking, sleep, appetite, energy and everyday functioning. It is different from short-lived sadness and can be treated.

2. Causes and Risk Factors

- Depression can involve genetic, biological, environmental and psychological factors.
- Stressful life events, other mental-health conditions and some medical conditions can contribute.
- Depression can occur at any age and may look different across age groups.

3. Common Symptoms

- Persistent sad, anxious or empty mood.
- Loss of interest or pleasure in activities.
- Hopelessness, guilt, worthlessness, irritability or restlessness.
- Fatigue or low energy and difficulty concentrating or making decisions.
- Sleep changes, including insomnia or oversleeping.
- Changes in appetite or unplanned weight change.
- Physical aches or digestive problems without a clear physical cause.
- Thoughts of death or suicide can occur and require urgent professional attention.

4. Diagnosis

- A clinician assesses symptoms, duration, severity and impact on daily life.
- For major depression, symptoms are present most of the day, nearly every day, for at least 2 weeks, with depressed mood or loss of interest/pleasure among the key features.
- A healthcare provider may also assess medical conditions or medicines that can produce similar symptoms.

5. Treatment

- Psychotherapy (talk therapy) is an established treatment; examples include cognitive behavioral therapy (CBT) and interpersonal therapy.
- Antidepressant medication may be used, especially for moderate or severe depression.
- Psychotherapy, medication, or a combination may be appropriate depending on the person and severity.
- Brain-stimulation treatments such as electroconvulsive therapy (ECT) or repetitive transcranial magnetic stimulation (rTMS) may be considered when other treatments have not worked or when severe illness requires them.

6. Self-Care and Support

- Maintain regular sleep and healthy meals and try to stay physically active as able.
- Stay connected with trusted people and follow the treatment plan recommended by a healthcare professional.
- Avoid using alcohol or drugs as a way to manage symptoms.

7. Urgent Help

- Thoughts of suicide, suicide attempts, or feeling unable to stay safe require immediate professional/emergency help.

Official reference sources
NIMH (National Institute of Mental Health), NIH — Depression, revised 2024: https://www.nimh.nih.gov/health/publications/depression
NIMH official downloadable PDF is available from the Depression publication page.

Educational use: This is a concise health-information summary for project/educational use. It does not replace diagnosis or individualized medical advice. Treatment decisions should be made by a qualified healthcare professional.

VIRAL HEPATITIS A/B/C

Clean disease factsheet | Overview, transmission, symptoms, treatment & prevention

1. Overview

Viral hepatitis A, B and C are infections caused by different hepatitis viruses that affect the liver. Hepatitis A usually resolves on its own; hepatitis B may become chronic; and hepatitis C commonly becomes chronic unless treated.

2. Transmission

- Hepatitis A is mainly spread by consuming food or water contaminated with infected feces and through close contact with an infected person.
- Hepatitis B spreads through infected blood or sexual fluids, including sharing injection equipment, contaminated tattoo/body-piercing equipment, needlestick exposure, and transmission from an infected mother to a newborn during childbirth.
- Hepatitis C is mainly spread through infected blood, including sharing injection equipment and contaminated tattoo/body-piercing tools; mother-to-child transmission can occur.

3. Common Symptoms

- Many people have no symptoms, especially with hepatitis C.
- Possible symptoms include fatigue, fever, nausea, vomiting, loss of appetite and stomach/abdominal discomfort.
- Jaundice (yellow skin or eyes), dark urine and light-colored stools may occur, particularly with hepatitis A or B.

4. Diagnosis

- Diagnosis requires appropriate blood testing for hepatitis-specific markers and/or viral material.
- Testing is particularly important when exposure has occurred or symptoms/risk factors suggest hepatitis.

5. Treatment

- Hepatitis A generally needs supportive care such as rest and fluids and usually resolves without chronic infection.
- Chronic hepatitis B may require ongoing medical monitoring and antiviral treatment when indicated.
- Hepatitis C can usually be cured with oral antiviral medicines prescribed by a clinician.

6. Complications

- Persistent hepatitis B or C can damage liver cells and may lead to cirrhosis (scarring) and liver cancer.
- Hepatitis C can recur as a new infection after cure because previous infection does not provide reliable immunity.

7. Prevention

- Hepatitis A: hand hygiene, safe food/water practices and vaccination.
- Hepatitis B: vaccination, safer sex, avoiding shared needles/razors/toothbrushes, and preventing blood exposure.
- Hepatitis C: avoid blood exposure and sharing injection or personal equipment; there is currently no vaccine for hepatitis C.

Official reference sources
U.S. Department of Veterans Affairs, Veterans Health Administration — The ABCs of Hepatitis Fact Sheet, updated May 2025: https://www.hepatitis.va.gov/HEPATITIS/pdf/ABC-of-hepatitis.pdf
Official VA Hepatitis website: https://www.hepatitis.va.gov/

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HIV/AIDS

Clean disease factsheet | Overview, transmission, symptoms, diagnosis, treatment & prevention

1. Overview

HIV (human immunodeficiency virus) attacks CD4 cells, which help the immune system fight infections. If untreated, HIV can progressively damage the immune system and may advance to AIDS, the most advanced stage of HIV infection.

2. Transmission

- HIV can be transmitted through certain body fluids including blood, semen, vaginal fluids, rectal fluids and breast milk.
- Common routes include vaginal or anal sex, sharing needles/syringes, blood exposure and transmission during pregnancy, childbirth or breastfeeding.
- HIV is not spread by casual contact such as hugging, shaking hands, sharing toilets or dishes, or by air, water or mosquitoes.

3. Symptoms

- Early HIV infection may cause flu-like symptoms such as fever or chills in some people.
- Other possible symptoms include night sweats, muscle aches, sore throat, fatigue, swollen lymph nodes, headache, diarrhea, mouth ulcers and skin rash.
- Some people have no symptoms during early infection.
- Without treatment, advanced HIV/AIDS can cause severe immune weakness and opportunistic infections.

4. Diagnosis

- HIV is diagnosed using appropriate laboratory testing rather than symptoms alone.
- An AIDS diagnosis can be based on a CD4 count below 200 cells/mm^3 or certain AIDS-defining opportunistic infections/conditions.

5. Treatment

- Antiretroviral therapy (ART) is recommended for everyone with HIV.
- ART reduces the amount of HIV in the blood, protects the immune system and helps prevent progression to AIDS.
- ART does not cure HIV, so treatment is continued as prescribed and monitored by healthcare professionals.

6. Prevention

- Use condoms correctly and consistently and avoid sharing needles or other injection equipment.
- PrEP can reduce the risk of acquiring HIV for people with ongoing risk.
- PEP may help prevent infection after a significant exposure when started promptly, generally within 72 hours.
- People with HIV who take ART as prescribed can greatly reduce sexual transmission risk.

7. Key Point

A person can have HIV without symptoms. Testing is the only way to know HIV status, and effective treatment allows people with HIV to live long, healthy lives.

Official reference sources
NIH HIVinfo — HIV and AIDS: The Basics, last reviewed May 28, 2025: https://hivinfo.nih.gov/understanding-hiv/fact-sheets/hiv-and-aids-basics
NIH HIVinfo — HIV/AIDS Fact Sheets: https://hivinfo.nih.gov/understanding-hiv/fact-sheets

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LYMPHATIC FILARIASIS

Clean disease factsheet | Overview, transmission, symptoms, treatment & prevention

1. Overview

Lymphatic filariasis (LF) is a parasitic disease caused by microscopic filarial worms. It is transmitted by infected mosquitoes. Chronic infection can damage the lymphatic system and cause long-term swelling and disability.

2. Transmission

- LF is spread through the bite of an infected mosquito.
- A mosquito becomes infected after feeding on a person who has filarial parasites and can later transmit the parasites to another person.

3. Symptoms and Clinical Features

- Many infected people may have no obvious symptoms for a long time.
- Chronic lymphatic damage can cause lymphedema, with severe swelling of the arms or legs.
- Elephantiasis refers to severe enlargement and hardening of affected body parts.
- Hydrocele is swelling of the scrotum in men and is an important chronic manifestation.
- These chronic manifestations can develop years after infection and may substantially affect quality of life.

4. Diagnosis

- Diagnosis requires appropriate laboratory assessment for filarial infection, using tests selected according to the clinical and epidemiological setting.
- Public-health surveillance and appropriate diagnostic tools are important for elimination programmes.

5. Treatment and Morbidity Management

- Preventive chemotherapy through mass drug administration (MDA) is used in endemic populations to reduce transmission.
- People with chronic lymphedema benefit from ongoing skin care, hygiene, limb care and disability-prevention measures.
- Hydrocele and other chronic complications may require clinical or surgical management according to individual assessment.

6. Prevention

- Mass drug administration in endemic areas is a major public-health strategy to interrupt transmission.
- Reducing mosquito bites can lower exposure risk, including use of bed nets, protective clothing and mosquito-control measures where appropriate.
- Early detection and access to morbidity-management services help prevent disability.

7. Key Point

The visible chronic manifestations of LF may appear years after infection. Elimination programmes therefore combine preventive treatment, surveillance and long-term care for people with lymphatic damage.

Official reference sources

CDC — Lymphatic Filariasis Factsheet, 2022: https://www.cdc.gov/parasites/resources/pdf/lf_factsheet.pdf

CDC Stacks — Lymphatic Filariasis Factsheet (FactSheet): https://stacks.cdc.gov/view/cdc/152717

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